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The Respected Editorial Board of *JNCI: Journal of the National Cancer Institute*

RE: Submission of Original Article – "Bladder Cancer in the United States: National Trends and State-Level Patterns from Global Burden of Disease (GBD) Study, 1980-2021"

Dear Editors,

We are pleased to submit our research manuscript, entitled **"Bladder Cancer in the United States: National Trends and State-Level Patterns from Global Burden of Disease (GBD) Study, 1980-2021,"** for consideration for publication as an Article in *JNCI: Journal of the National Cancer Institute*.

Bladder cancer remains a significant public health challenge in the United States, yet comprehensive evaluations of its long-term burden at both national and state levels are limited. In this study, we leveraged robust, population-based estimates from the Global Burden of Disease (GBD) 2021 study to characterize temporal trends in prevalence, incidence, mortality, Years Lived with Disability (YLDs), and disability-adjusted life years (DALYs) across the U.S. over four decades (1980–2021). We further examined state-level disparities and benchmarked U.S. trends against other major health systems, including the European Union and high-income countries.

Significance of Our Findings Our analysis reveals that while some burden metrics have improved since 1990, bladder cancer continues to impose a substantial and uneven burden across U.S. states. We identified persistent state-level variations and significant gender disparities. These insights provide a critical evidence base for targeted prevention strategies, early detection efforts, and health policy development, with potential implications for optimizing healthcare spending by prioritizing prevention over late-stage treatment.

Key Strengths The primary strength of our study lies in its use of standardized, population-based GBD estimates to examine long-term trends and state-level disparities. Our analytical workflow is **fully transparent and reproducible**; all data, code, and supplementary visualizations, including geofacet plots, animated population pyramids, and state ranking bump plots, are publicly accessible via our GitHub repository and the Global Health Data Exchange (GHDx).

Transparent Consideration of Limitations We acknowledge several limitations, including the reliance on modeled data due to incomplete primary sources, which may introduce uncertainty, particularly in states with sparse reporting. The inherent lag in GBD data availability precludes analysis beyond 2021, and our descriptive design does not permit causal inference regarding individual-level risk factors. Additionally, as our estimates are age-standardized, discrepancies with surveillance systems reporting crude or partially adjusted rates reflect methodological differences rather than true epidemiological disagreement. These limitations are thoroughly addressed in our Discussion.

Manuscript Specifications

- **Manuscript Type:** Article (Original Research)
- **Word Count:** ~ 3,700 words (excluding abstract, references, tables, and figure legends)
- **Figures/Tables:** 5 figures and 0 tables
- **Supplementary Materials:** Supporting figures and datasets are available via GitHub

Declarations

- **Transparency Statement:** The lead author, Alireza Sadeghi, affirms that this manuscript is an honest, accurate, and transparent account of the study being reported; that no important aspects of the study have been omitted; and that any discrepancies from the study as planned have been explained.
- **Data Availability:** All data are publicly available through the Global Health Data Exchange (GHDx). The complete analytic workflow, including cleaned datasets and all code required to reproduce the analyses, figures, and tables presented in this manuscript, is available in the study's GitHub repository.
- **Conflicts of Interest:** The authors declare no conflicts of interest.
- **Prior Publication:** This work has not been previously published and is not under consideration for publication elsewhere. All authors have approved the manuscript and agree with its submission to *JNCI*.

We believe our findings will be of broad interest to the *JNCI* readership, including epidemiologists, clinicians, and health policy makers, and are confident that this work aligns with the Journal's mission to disseminate high-impact research on cancer prevention, treatment, and control.

We appreciate your consideration of our manuscript and look forward to your response. Please direct all correspondence regarding this submission to the corresponding author at the address below.

Sincerely,

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